

7.35.3 NMAC: proposed amendments to the twenty-four sections outside the practicum

Working draft v3, July 26, 2026. Rule hearing August 28, 2026. Not a filing, not submitted, and not adopted rule text.

- **Twenty-three provisions are amended.** One redline, sections in numerical order. Every change is a mechanical correction, a conforming amendment between two provisions of the same rule, or a restoration of wording the July 9, 2026 draft carried. No figure or policy choice enters without a source.
- **Four provisions are reproduced without amendment.** Each carries a question. Every question in the document sits in an orange callout at its provision, question first, and all of them are on one page in Addendum E, grouped by who answers.
- **Eleven further defects are recorded and not drafted.** Addendum C lists each with the reason.
- Questions are limited to gaps, contradictions, typos, and drafting mistakes. The unfilled dates and placeholders are the department's to fill at publication and are not raised. Nothing here reopens the practicum draft: read against its working draft v9, and Addendum B records every dependency between the two scopes.

The columns. Left is the rule as published, verbatim. Right is the proposed amendment: underlined green inserted, ~~struck red deleted~~, unmarked text carried forward unchanged. A badge such as

90 from 7.35.3.11 D

 names the provision a figure is taken from. The source note and, where there is one, the question run the full width beneath both columns.

Contents

WHERE	WHAT
The redline	Sections .7, .8, .9, .10, .11, .12, .13, .16, .17, .23, .24, .25, .27, in numerical order.
Addendum A	Terms this part uses and no part defines, counted in both parts, with the material for a definition of each assembled from the sources.
Addendum B	Dependencies between these sections and the practicum draft, in both directions.
Addendum C	Every defect recorded and not drafted, with the reason nothing is proposed.
Addendum D	Numbering and drafting-form defects across all twenty-eight sections.
Addendum E	Every question in the document, one line each, grouped by who answers. One page.

7.35.3.7 Definitions

CURRENT LANGUAGE	PROPOSED AMENDMENT
Proposed rule as published July 23, 2026	Draft for review. Not filed, not adopted
Entire section, definitions 7.35.3.7 DEFINITIONS: The definitions in 7.35.2.7 NMAC apply to this part.	Entire section, definitions 7.35.3.7 DEFINITIONS: <u>A</u>. The definitions in 7.35.2.7 NMAC apply to this part. <u>B. In addition to the definitions in 7.35.2.7 NMAC, the following definitions apply to this part:</u> <u>(1) “Certificant” means a practitioner, certifying clinician, facilitator, healing center or other approved location, or educational program that holds a certification issued by the department under this part.</u> <u>(2) “Facilitator” means an individual who is certified by the department under this part to</u>

work alongside a practitioner during medical psilocybin services, under the direct supervision of a practitioner, and to provide peer support to qualified patients as well as logistical and administrative support to the practitioner and to the healing center.
(3) “Other approved location” means a physical location at which medical psilocybin is intended to be consumed, that is not the location of a healing center, and that holds a temporary certification issued by the department under Subsection B of 7.35.3.11 NMAC.
(4) “Practicum” means the supervised practice training required of an individual who seeks to become certified as a practitioner or facilitator under 7.35.3.19 NMAC.
(5) “Registrant of another approved location” means the practitioner or facilitator who holds a certification of an other approved location issued under Subsection B of 7.35.3.11 NMAC.
(6) “Student” means an individual who is registered with a psilocybin educational program certified by the department under this part.

SOURCE Six definitions are added. Each is drawn from a single provision of the rule as published, and none is composed. Certificant: 7.35.3.26 NMAC, page 16. Facilitator: 7.35.3.13 NMAC Subsection B, page 8, with 7.35.3.9 NMAC Subsection F, page 3. Other approved location: 7.35.3.11 NMAC Subsection B, page 6. Practicum: 7.35.3.19 NMAC Subsection A, page 12; that section is the practicum draft's, whose v9 keeps the defining phrase while amending the hour figures, and Addendum B records the dependency. Registrant of another approved location: 7.35.3.11 NMAC Subsection B, page 6. Student: 7.35.3.20 NMAC Paragraph (5) of Subsection H, page 14. As published the section reads in full: “The definitions in 7.35.2.7 NMAC apply to this part.” Amending 7.35.2 NMAC, adopted effective June 23, 2026, is a separate rulemaking, so any term this part needs has to be carried here.

Question. Which of the ten undefined terms should the department be asked to define? Healing center, 54 uses, and certifying clinician, 53, lead; Addendum A assembles the material for each. Does the Act's exemption cover facilitators? Section 5 of the Act exempts a producer, a clinician and a qualified patient; a facilitator is none of these, and 7.35.3.14 B lets facilitators possess and provide medical psilocybin. The Training and Education Committee picks the slate; Department of Health staff and department counsel answer the exemption question.

7.35.3.8 Patient enrollment application process

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection D, review of a denied patient application

D. A person whose application for patient enrollment is denied for failure to complete an application or failure to meet a submittal requirement of this rule may request a record review to be conducted by the department, in accordance with this rule.

Subsection D, review of a denied patient application

D. A person whose application for patient enrollment is denied for failure to complete an application or failure to meet a submittal requirement of this rule may request ~~a record review to be conducted by the department~~ an informal administrative review under 7.35.3.25 NMAC, in accordance with this rule.

SOURCE Conforming amendment. 7.35.3.8 NMAC Subsection D, page 2, gives a denied patient applicant "a record review to be conducted by the department". The rule provides no proceeding called a record review. 7.35.3.25 NMAC, page 16, provides an informal administrative review for "An applicant for patient enrollment whose application has been denied", which is the same class of person. The amendment names that proceeding and cites it. The July 9, 2026 draft titled the corresponding section "DENIAL OF AN INITIAL PATIENT APPLICATION; RECORD REVIEW", and its Subsection A read that applicants "may request a record review from the department", so the two names described one proceeding in that draft. Section 7.35.3.8 is new in the published rule.

Question. Which denials get the informal review, and which the hearing? Three provisions overlap: this subsection covers incomplete applications, 7.35.3.25 A covers any denial, and 7.35.3.27 C(7) covers every denial except incomplete applications, so a patient denied on the merits may have both tracks. The amendment fixes only the name of the proceeding. Department of Health staff and department counsel align the scopes.

7.35.3.9 Certifying clinician, practitioner, and facilitator application process**CURRENT LANGUAGE**

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Paragraphs (5) and (6) of Subsection A, re-application after denial

(5) If the application is denied, the applicant may re-apply within six months. (6) An applicant who is denied a second time may not re-apply for six months from the denial.

Paragraphs (5) and (6) of Subsection A, re-application after denial

(5) If the application is denied, the applicant may re-apply ~~within~~ no earlier than six months after the date of the denial. (6) An applicant who is denied a second time may not re-apply for six months from the denial.

SOURCE Conforming amendment. Same defect and same fix as 7.35.3.10 NMAC Subsection C. 7.35.3.9 NMAC Paragraphs (5) and (6) of Subsection A, page 2.

Subsection D, certifying clinician application requirements

D. Certifying clinician application requirements: An applicant for certification as a certifying clinician shall additionally submit the following, using the electronic system designated by the department: (1) Documentation of current professional license to practice in New Mexico (e.g. MD, NP); (2) NM controlled substance number; (4) Documentation of HIPAA certification completed within the preceding two years; (5) Completed W-9 form; and (6) Such additional information as the department may reasonably require.

Subsection D, certifying clinician application requirements

D. Certifying clinician application requirements: An applicant for certification as a certifying clinician shall additionally submit the following, using the electronic system designated by the department: (1) Documentation of current professional license to practice in New Mexico (e.g. MD, NP); (2) NM controlled substance number; ~~(4)~~ **(3)** Documentation of HIPAA certification completed within the preceding two years; ~~(5)~~ **(4)** Completed W-9 form; and ~~(6)~~ **(5)** Such additional information as the department may reasonably require.

SOURCE Correction. Finding N3 of `analysis/july23-rule-concerns.md`, verified against the published rule. The paragraphs of 7.35.3.9 NMAC Subsection D, page 3, run (1), (2), (4), (5), (6). There is no (3). The amendment renumbers so that the list is consecutive and adds no item. The paragraph reproduced here contains the controlled substance number requirement at Paragraph (2), which is out of scope for this document by decision; that paragraph is reproduced as published and is not amended.

Question. Numbering error, or a dropped requirement? The paragraphs run (1), (2), (4), (5), (6); there is no (3), and the section has no July 9 counterpart to show what stood there. The renumbering drafted here assumes an error. Department of Health staff confirm nothing was meant to fill the gap.

7.35.3.10 Application process based on educational programs from other jurisdictions**CURRENT LANGUAGE**

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Paragraph (2) of Subsection B, documentation of equivalency

(2) Documentation of either: (a) Completion of a government approved psilocybin educational program from another jurisdiction, which shall include: (i) Documentation that the curriculum of the educational program is equivalent to the didactic educational requirements applicable for New Mexico certification; and (ii) For all applications received on or after January 1, 2028, documentation of a third-party evaluation of the curriculum demonstrating such equivalency; (b) Completion of a psilocybin educational program from another jurisdiction that is developed in collaboration with higher education institutions or government-approved psychedelic research studies, which shall include: (i) Documentation that the curriculum of the educational program is equivalent to the didactic educational requirements applicable for New Mexico certification, and (ii) For all applications received on or after January 1, 2028, documentation of a third-party evaluation of the curriculum demonstrating such equivalency; or (c) Completion of an educational program that is currently included on the list of department-approved psilocybin educational programs from other jurisdictions.

Paragraph (2) of Subsection B, documentation of equivalency

(2) Documentation of either: (a) Completion of a government approved psilocybin educational program from another jurisdiction, which shall include: (i) Documentation that the curriculum of the educational program is equivalent to the didactic educational requirements applicable for New Mexico certification; and (ii) For all applications received on or after January 1, 2028, documentation of a third-party evaluation of the curriculum demonstrating such equivalency, except that an applicant may satisfy this item by documenting that the educational program is included on the list maintained under Subsection A of this section; (b) Completion of a psilocybin educational program from another jurisdiction that is developed in collaboration with higher education institutions or government-approved psychedelic research studies, which shall include: (i) Documentation that the curriculum of the educational program is equivalent to the didactic educational requirements applicable for New Mexico certification, and (ii) For all applications received on or after January 1, 2028, documentation of a third-party evaluation of the curriculum demonstrating such equivalency, except that an applicant may satisfy this item by documenting that the educational program is included on the list maintained under Subsection A of this section; or (c) Completion of an educational program that is currently included on the list of department-approved psilocybin educational programs from other jurisdictions.

SOURCE Conforming amendment. Items (a)(ii) and (b)(ii) of 7.35.3.10 NMAC Paragraph (2) of Subsection B, page 4, require an individual applicant to produce a third-party evaluation of a curriculum. Under 7.35.3.16 NMAC Subsection A, page 10, a third-party evaluation is commissioned by a psilocybin educational program, not by an individual. Item (c) of the same paragraph already lets an applicant rely on the list maintained under Subsection A, and this amendment extends that route to items (a)(ii) and (b)(ii). No figure is added.

Subsection C, re-application after denial

C. Application approval and denial; certification period: If the application is approved, the certification shall be effective on the date of notice and shall be valid for two years. If the department denies an application, the department shall provide notice of the denial in accordance with this rule. If the application is denied, the applicant may re-apply within six months. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

Subsection C, re-application after denial

C. Application approval and denial; certification period: If the application is approved, the certification shall be effective on the date of notice and shall be valid for two years. If the department denies an application, the department shall provide notice of the denial in accordance with this rule. If the application is denied, the applicant may re-apply ~~within~~ no earlier than six months after the date of the denial. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

SOURCE Conforming amendment, one of four in the rule. 7.35.3.10 NMAC Subsection C, page 4, reads “the applicant may re-apply within six months”, which on its face sets a deadline. The next sentence of the same subsection reads “An applicant who is denied a second time may not re-apply for six months from the denial”, which sets a waiting period. The amendment conforms the first sentence to the second. The same pair appears at 7.35.3.9 NMAC Paragraphs (5) and (6) of Subsection A, page 2; 7.35.3.11 NMAC Subsection D, page 7; and 7.35.3.12 NMAC Subsection D, page 8. All four are drafted the same way in this document. No figure is added.

Question. Is the six months after a first denial a waiting period or a deadline? This subsection says a denied applicant “may re-apply within six months”, then bars a second-time applicant “for six months from the denial”. The amendment reads both as waiting periods, and the same fix is drafted at 7.35.3.9 A, 7.35.3.11 D and 7.35.3.12 D. Department of Health staff confirm the intended reading.

Paragraph (1) of Subsection D, waiver of practicum requirements

D. Waiver of practicum requirements: (1) An applicant for certification as a practitioner or facilitator on the basis of an educational program from another jurisdiction, whose application is received by the department by December 31, 2027, shall not be required to complete the full practicum hours required by New Mexico, but shall demonstrate completion of at least 40 hours of contact time with a minimum of two separate individual sessions and two separate group sessions for preparation, administration, and integrative therapy with psilocybin treatment.

Paragraph (1) of Subsection D, waiver of practicum requirements

Not amended. Reproduced for the question below.

SOURCE Not amended. Reproduced, at page 5, for the conflict with Paragraph (4) of Subsection G of 7.35.3.19 NMAC, page 13, stated in the callout below. Addendum B and Addendum C carry the cross-references.

Question. Two group sessions or one, inside the same 40 waiver hours? This paragraph requires “two separate group sessions”; 7.35.3.19 G(4) accepts “A minimum of one group session”, and an out-of-jurisdiction applicant whose program is on the department list can qualify under both at once. The July 9, 2026 draft read “or two separate group sessions” in both places, so no restoration conforms them. The Training and Education Committee chooses; the practicum draft records the same conflict and amends neither.

7.35.3.11 Application process for healing centers and other approved locations

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading and lead-in of Subsection A

7.35.3.11 APPLICATION PROCESS FOR HEALING CENTERS AND OTHER APPROVED LOCATIONS: (A) Applications for healing centers: An applicant seeking certification as a healing center shall submit a complete application through the electronic system designated by the department. A complete application shall include, at minimum, the following:

Section heading and lead-in of Subsection A

7.35.3.11 APPLICATION PROCESS FOR HEALING CENTERS AND OTHER APPROVED LOCATIONS: ~~(A)~~ A. Applications for healing centers: An applicant seeking certification as a healing center shall submit a complete application through the electronic system designated by the department. A complete application shall include, at minimum, the following:

SOURCE Correction only. 7.35.3.11 NMAC Subsection A, page 5, is lettered “(A)”; Subsections B, C and D of the same section use the form “B.”, “C.” and “D.” The only other section of the rule that uses parenthetical subsection letters is 7.35.3.14 NMAC, page 9, which is addressed in the practicum amendment document. No substance is changed.

Paragraph (11) of Subsection A, owners and employees

(11) A list of all owners or members of the board of directors, including corresponding contact information;

Paragraph (11) of Subsection A, owners and employees

(11) A list of all owners or members of the board of directors, including corresponding contact information; and a list of all owners and employees whom the healing center designates to purchase, possess, sell, or otherwise administer medical psilocybin products under Subsection C of 7.35.3.14 NMAC, including corresponding contact information. An owner or employee named on the list submitted under this paragraph is registered with the department for purposes of Subsection C of 7.35.3.14 NMAC;

SOURCE Finding B5 of `analysis/july23-rule-concerns.md`, verified against the published rule. Subsection C of 7.35.3.14 NMAC, page 9, conditions the authority of healing center owners and employees to purchase, possess, sell or administer medical psilocybin on their being “registered with the department”. The phrase “registered with the department” appears once in 7.35.3 NMAC, at that provision, and no registration for owners or employees exists in 7.35.3 NMAC or in 7.35.2 NMAC. The amendment supplies the registration at the point where the healing center already files the names: Paragraph (11) of Subsection A already requires “A list of all owners or members of the board of directors, including corresponding contact information”, Paragraph (2) of the same subsection already requires “Names and contact information of primary contact person(s) and any affiliated practitioners or facilitators”, and Subsection C of 7.35.3.14 NMAC already requires that the individual be “designated to engage in each activity by the healing center”. Subsection C of 7.35.3.14 NMAC is new in the published rule; the July 9, 2026 draft contained no authorized-possession section. No figure is added.

Question. Is this the right home for the registration 7.35.3.14 C requires? That provision conditions healing center staff authority on being “registered with the department”, and no such registration exists in 7.35.3 NMAC or 7.35.2 NMAC. It is drafted here, where the healing center already files the names; the practicum draft assigns the fix to 7.35.3.11 or 7.35.3.20. Department of Health staff decide the home.

Paragraph (9) of Subsection B, proof of ownership

(9) Proof of ownership by the qualified patient or attending practitioner or facilitator of any property on which medical psilocybin administration sessions will be conducted, or a signed, written statement from the owner of such property acknowledging that the owner understands that persons will be participating in the medical psilocybin program on the premises, and what those persons are authorized to do within the terms of their certification;

Paragraph (9) of Subsection B, proof of ownership

(9) Proof of ownership by the qualified patient or attending practitioner or facilitator of any property on which medical psilocybin administration sessions will be conducted, or a signed, written statement from the owner of such property acknowledging that the owner understands that persons will be participating in the medical psilocybin program on the premises, and what those persons are authorized to do within the terms of their certification. This paragraph does not apply where the location is the residence of the qualified patient;

SOURCE Wilson working redline of July 25, 2026, comment on this paragraph: “seems like this will require EOL patients who are getting at home treatment to get permission from their landlord. We don't require that for any other in home healthcare and it's not appropriate to require that for EOL patients. People should not have to get their landlords permission to have certain healthcare.” End-of-life care is a qualifying condition under Section 3(l) of the Medical Psilocybin Act and under 7.35.2.7 NMAC. 7.35.3.11 NMAC Paragraph (9) of Subsection B, page 6. Carried over from the July 9, 2026 draft.

Question. Keep the owner's statement for a patient's own residence, or adopt the exemption drafted here? The Wilson redline records that the paragraph as published makes an end-of-life patient get a landlord's permission for treatment at home. The exemption removes that, and it also removes the only notice an owner gets. The board weighs access against notice.

Paragraph (10) of Subsection B, outdoor or natural environments

(10) For outdoor or natural environments: (a) A detailed description of the administration area, including identification of safe entrances and exits, and verification that the area is free from hazards; (b) An emergency safety and response plan; (c) Proof that emergency medical services can reliably be contacted from, and respond to, the location at which medical psilocybin administration sessions will occur; and (d) If the natural environment setting is 15 minutes or more away from emergency medical service response or a hospital, urgent care, or other emergency medical services, then there must be at least two individuals present who are not receiving treatment and who have: (i) Wilderness first aid certification; (ii) Wilderness first responder certification; or (iii) Licensure as a New Mexico emergency medical technician (e.g., EMT-basic, EMT-intermediate, EMT-paramedic);

Paragraph (10) of Subsection B, outdoor or natural environments

(10) For outdoor or natural environments: (a) A detailed description of the administration area, including identification of safe entrances and exits, and verification that the area is free from hazards; (b) An emergency safety and response plan; (c) Proof that emergency medical services can reliably be contacted from, and respond to, the location at which medical psilocybin administration sessions will occur; and (d) If the natural environment setting is 15 minutes or more away from emergency medical service response or a hospital, urgent care, or other emergency medical services, then there must be at least two individuals present who are not receiving treatment and who have: (i) Wilderness first aid certification; (ii) Wilderness first responder certification; or (iii) Licensure as a New Mexico emergency medical technician (e.g., EMT-basic, EMT-intermediate, EMT-paramedic); and (e) If the natural environment setting is 15 minutes or more away from emergency medical service response or a hospital, urgent care, or other emergency medical services, then there must be a basic first aid kit and an AED on site;

SOURCE Conforming amendment. Finding M16 of `analysis/july23-rule-concerns.md`, verified against the published rule. The outdoor requirements for a healing center at 7.35.3.11 NMAC Paragraph (22) of Subsection A, pages 5 and 6, run to item (e) and require “a basic first aid kit and an AED on site”. The outdoor requirements for an other approved location at Paragraph (10) of Subsection B of the same section, pages 6 and 7, stop at item (d). The added item is the text of item (e) of Paragraph (22) of Subsection A, reproduced. No figure is added; the 15 minutes is the figure already in both paragraphs.

Question. Where does the two-person requirement belong? The Wilson redline adds it at the healing center paragraph and records that “the 2 person universal requirement did not make it into the published rules, though DOH did confirm they mean for it to be universal.” Ms. Wilson and Department of Health staff place it.

Subsection D, certification period; approval and denial

D. Certification period; approval and denial: Certification of a healing center shall be effective on the date of department issuance and shall be valid for two years. Certification of other approved locations shall be effective on the date of department issuance and shall be valid for 90 days. If the department denies an application for a healing center or other approved location, the department shall provide notice of the denial within 30 calendar days in accordance with this rule. If the application is denied, the applicant may re-apply within six months. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

Subsection D, certification period; approval and denial

D. Certification period; approval and denial: Certification of a healing center shall be effective on the date of department issuance and shall be valid for two years. Certification of other approved locations shall be effective on the date of department issuance and shall be valid for 90 days, and may be renewed for additional 90-day terms on application to the department 90 from 7.35.3.11 D. If the department denies an application for a healing center or other approved location, the department shall provide notice of the denial within 30 calendar days in accordance with this rule. If the application is denied, the applicant may re-apply ~~within no~~ earlier than six months after the date of the denial. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

SOURCE Two changes. Renewal: finding M22 of `analysis/july23-rule-concerns.md`, verified against the published rule. Subsection D gives an other approved location a 90-day certification and no renewal route, while Subsection C of the same section supplies a renewal route for healing centers only. The 90 days in the amendment is the figure already in Subsection D. Re-application: the same conforming change as 7.35.3.10 NMAC Subsection C. 7.35.3.11 NMAC Subsection D, page 7. Carried over from the July 9, 2026 draft.

7.35.3.12 Application process for psilocybin educational programs

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection D, re-application after denial

D. Approval and denial; certification period: If the application is approved, the certification shall be effective on the date of notice and shall be valid for two years. If the department denies an application, the department shall provide notice of the denial in accordance with this rule. If the application is denied, the applicant may re-apply within six months. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

Subsection D, re-application after denial

D. Approval and denial; certification period: If the application is approved, the certification shall be effective on the date of notice and shall be valid for two years. If the department denies an application, the department shall provide notice of the denial in accordance with this rule. If the application is denied, the applicant may re-apply ~~within~~ **no earlier than** six months **after the date of the denial**. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

SOURCE Conforming amendment. Same defect and same fix as 7.35.3.10 NMAC Subsection C. 7.35.3.12 NMAC Subsection D, page 8. Carried over from the July 9, 2026 draft.

7.35.3.13 Requirements and prohibitions for certifying clinicians, practitioners, and facilitators

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading

7.34.3.13 REQUIREMENTS AND PROHIBITIONS FOR CERTIFYING CLINICIANS, PRACTITIONERS, AND FACILITATORS:

Section heading

~~7.34.3.13~~ **7.35.3.13** REQUIREMENTS AND PROHIBITIONS FOR CERTIFYING CLINICIANS, PRACTITIONERS, AND FACILITATORS:

SOURCE Correction only. Finding N1 of `analysis/july23-rule-concerns.md`, verified against the published rule. The heading at page 8 reads 7.34.3.13; the history note at the end of the same section reads “[7.35.3.13 NMAC, xx/xx/2026]”. Chapter 34 of Title 7 is a different chapter. Five headings in the rule read 7.34.3: those of 7.35.3.13, 7.35.3.14, 7.35.3.20, 7.35.3.23 and 7.35.3.25. The headings of 7.35.3.14 and 7.35.3.20 are corrected in the practicum amendment document; the other three are corrected here.

Subsection B, facilitators; scope of work

B. Facilitators; scope of work: A facilitator is authorized to work alongside a practitioner during medical psilocybin services. A facilitator works under the direct supervision of a practitioner, and provides peer support to qualified patients, as well as logistical and administrative support to the practitioner and to the healing center. A facilitator shall not perform any patient care outside this scope, unless another license held by the facilitator permits it.

Subsection B, facilitators; scope of work

Not amended. Reproduced for the question below.

SOURCE Not amended. Recorded because it is the source of the facilitator definition drafted at 7.35.3.7 NMAC and because of finding M15 of `analysis/july23-rule-concerns.md`. 7.35.3.13 NMAC Subsection B, page 8. Carried over from the July 9, 2026 draft.

Question. Does direct supervision mean in the room, at the location, or available? This subsection limits a facilitator to peer support under a practitioner's "direct supervision", while 7.35.3.14 B lets facilitators possess and provide psilocybin and 7.35.3.20 H(5) lets a facilitator be the only certificant with a patient in a group session. The rule never defines the term. The Training and Education Committee and Department of Health staff say what it means; the conflicting provisions are in the practicum draft's scope.

7.35.3.16 Requirements for third-party evaluators of educational programs**CURRENT LANGUAGE**

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading and Subsection A, general requirements

7.35.3.16 REQUIREMENTS FOR THIRD-PARTY EVALUATORS OF EDUCATIONAL PROGRAMS: A. General requirements: A psilocybin educational program shall engage a qualified third-party evaluation team to conduct the evaluations required under this rule. The evaluation team shall consist of no fewer than three individuals who collectively meet all competency requirements set forth in this section.

Section heading and Subsection A, general requirements

7.35.3.16 REQUIREMENTS FOR THIRD-PARTY EVALUATORS OF EDUCATIONAL PROGRAMS: A. General requirements: A psilocybin educational program shall engage a qualified third-party evaluation team to conduct the evaluations required under this rule. The evaluation team shall consist of no fewer than three individuals who collectively meet all competency requirements set forth in this section. A program may compensate the evaluation team for conducting an evaluation required under this rule, and that compensation is not a conflict of interest under Subsection C of this section.

SOURCE Finding B3 of `analysis/july23-rule-concerns.md`, verified against the published rule. 7.35.3.16 NMAC Subsection A, page 10, requires that a program "shall engage a qualified third-party evaluation team". Item (b) of Paragraph (2) of Subsection C of the same section, page 10, disqualifies an evaluator for "receiving compensation from... the program or organization being evaluated", with no exception for the engagement itself. The amendment states that the engagement fee is not a disqualifying conflict. Both provisions are carried over from the July 9, 2026 draft.

Question. Adopt the engagement-fee cure at Subsection A, at C(2)(b), or both? As published no evaluator can serve: a program "shall engage a qualified third-party evaluation team", and C(2)(b) disqualifies anyone "receiving compensation from... the program or organization being evaluated". Both cures are drafted. The Training and Education Committee chooses.

Paragraph (2) of Subsection B, minimum qualifications

B. Minimum qualifications: Each evaluator shall possess at least three years of professional experience in one or more of the following fields, and the evaluation team as a whole shall demonstrate expertise covering all listed domains: (1) Psilocybin therapy practice, supported by a master's or doctoral degree in counseling, therapy, social work, public health, behavioral health, or a closely related field; (2) Medical and research practice, supported by a master's or doctoral degree in medicine, pharmacy, nursing, or another related clinical or scientific field; and (3) Educational curriculum development and evaluation, supported by a master's or doctoral degree in education, educational psychology, or a closely related field.

Paragraph (2) of Subsection B, minimum qualifications

B. Minimum qualifications: Each evaluator shall possess at least three years of professional experience in one or more of the following fields, and the evaluation team as a whole shall demonstrate expertise covering all listed domains: (1) Psilocybin therapy practice, supported by a master's or doctoral degree in counseling, therapy, social work, public health, behavioral health, or a closely related field; (2) Medical ~~and~~ or research practice, supported by a master's or doctoral degree in medicine, pharmacy, nursing, or another related clinical or scientific field; and (3) Educational curriculum development and evaluation, supported by a master's or doctoral degree in education, educational psychology, or a closely related field.

SOURCE Restoration. The July 9, 2026 draft read "Medical or research practice". The published rule at 7.35.3.16 NMAC Paragraph (2) of Subsection B, page 10, reads "Medical and research practice". On the published wording an evaluator in this domain needs both medical and research practice; on the July 9 wording either will do. The amendment restores the July 9 wording.

Question. Was the change from or to and deliberate? The July 9, 2026 draft read "Medical or research practice"; the published rule reads "Medical and research practice", so an evaluator in this domain now needs both. The amendment restores the July 9 reading. Department of Health staff confirm the intent.

Subsection C, conflict of interest prohibitions

C. Conflict of -interest prohibitions: (1) All evaluators shall be free from actual or perceived conflicts of interest regarding the organization or curriculum under review. (2) The following conflicts, at minimum, shall disqualify an individual from serving as an evaluator: (a) Participation in the design, development, or teaching of the curriculum being evaluated; or (b) Holding a financial interest in, receiving compensation from, or serving in any governance or advisory capacity with the program or organization being evaluated.

Subsection C, conflict of interest prohibitions

C. Conflict ~~of -interest~~ of-interest prohibitions: (1) All evaluators shall be free from actual or perceived conflicts of interest regarding the organization or curriculum under review. (2) The following conflicts, at minimum, shall disqualify an individual from serving as an evaluator: (a) Participation in the design, development, or teaching of the curriculum being evaluated; or (b) Holding a financial interest in, receiving compensation from, or serving in any governance or advisory capacity with the program or organization being evaluated, other than compensation for conducting an evaluation required under this rule as provided in Subsection A of this section.

SOURCE Two changes. Correction: the heading at 7.35.3.16 NMAC Subsection C, page 10, reads "Conflict of -interest prohibitions"; the July 9, 2026 draft read "Conflict-of-interest prohibitions". Substance: the carve-out for the engagement fee, consequential on the amendment to Subsection A of the same section. Finding B3.

7.35.3.17 Educational programs; mentoring requirements; record-keeping**CURRENT LANGUAGE**

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection A, mentoring sessions

A. All educational programs shall provide each practitioner and facilitator student with a minimum of 10 hours of mentoring sessions after graduation and after practicum hours are completed. The purpose of the mentoring sessions is to allow new graduates to consult with the educational program faculty regarding cases, patients, situations, and how they handle them. These mentoring sessions may not have additional associated charges beyond the cost of the educational program itself. The 10 hours are valid for up to 12 months after practicum hours are

Subsection A, mentoring sessions

Not amended. Reproduced for the question below.

completed. If students request more than 10 hours of mentoring sessions, this will be at the purview of the educational program and may have an additional associated cost.

SOURCE Not amended. 7.35.3.17 NMAC Subsection A, pages 10 and 11, carried over from the July 9, 2026 draft. The Wilson working redline of July 25, 2026 rewrites this subsection: it retitles the section heading to read "CASE-CONSULTATION" in place of "MENTORING", replaces the 10 hours with "2 student case" consultations, adds that "Case presentations under consultation shall include discussion of individual case concerns, risk factors, supportive factors, treatment considerations, and recommendations for aftercare.", and strikes the last two sentences of the subsection. Her comment on the strike reads "want to keep this in or strike it? Would change hours to cases." and her comment on the added sentence reads "This probably does not need to live in regulation but can be part of a program guide for educational programs DOH could host and update regularly". This document draws no figure and no text from the redline here, because the placement of the consultation requirement is an open question in the practicum amendment document.

Question. Hours or cases, and stated here or at the practicum draft's proposed 7.35.3.19 H? Open with Dr. Metz, Ms. Wilson and Dr. Leeman; as published, 7.35.3.19 runs A through G and has no Subsection H. One warning while it is open: the 10 mentoring hours here run "after graduation and after practicum hours are completed" and sit inside no program total, so if the consultation requirement lands at 7.35.3.19 H while this subsection stands, a graduate owes both sets of hours.

Subsection B, module evaluation without attendance

B. Educational programs shall offer an option for a student to complete the evaluation for each educational module (not including the New Mexico module) without attending the lessons or classes for the module. The cost to complete the evaluation for each module shall not be greater than one-fourth of the normal price of each of the educational modules. If the student receives a passing grade for a module evaluation, they will have been considered to have completed the module; and if a student does not receive a passing grade for a module evaluation, they will not have completed the module. The educational program may at its discretion allow students to re-take the evaluation without attending the lessons or classes for the modules. Students who utilize this option shall complete all other requirements for the educational program, including the New Mexico Module, certifications, simulated patient, and practicum requirements to graduate from the educational program.

Subsection B, module evaluation without attendance

Not amended. Reproduced for the question below.

SOURCE Not amended. 7.35.3.17 NMAC Subsection B, page 11, carried over from the July 9, 2026 draft. The Wilson working redline of July 25, 2026 strikes the whole subsection, and her comment on it reads "Strike, right? I don't think we want this test out provision."

Question. Strike the test-out option, or keep it and restate the cap? The Wilson redline strikes the subsection: "Strike, right? I don't think we want this test out provision." If it stays, the cap is broken for any program with a single price, because it is a fraction of the "normal price of each of the educational modules". The Training and Education Committee decides whether it survives.

7.35.3.23 Prohibition against dual ownership in certificant and permittee

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Entire section, dual ownership

7.34.3.23 PROHIBITION AGAINST DUAL OWNERSHIP IN CERTIFICANT AND PERMITTEE: A certificant or a person who holds an ownership interest in a certificant shall not hold an ownership interest in a permittee.

Entire section, dual ownership

~~7.34.3.23~~ **7.35.3.23** PROHIBITION AGAINST DUAL OWNERSHIP IN CERTIFICANT AND PERMITTEE: A certificant or a person who holds an ownership interest in a certificant shall not hold an ownership interest in a permittee.

SOURCE Correction only. Finding N1. The heading at page 15 reads 7.34.3.23; the history note at the end of the same section reads "[7.35.3.23 NMAC, xx/xx/2026]". The substance is not amended. "Permittee" is defined at 7.35.2.7 NMAC as a psilocybin producer or psilocybin testing laboratory that holds a permit, so the prohibition operates on the producer and laboratory side without further definition. Carried over from the July 9, 2026 draft.

7.35.3.24 Complaints to the department**CURRENT LANGUAGE**

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Entire section, complaints to the department

7.35.3.24 COMPLAINTS TO THE DEPARTMENT: A qualified patient or certificant may submit a complaint to the department regarding a certificant in the medical psilocybin program, or regarding any activity or concern related to the medical psilocybin program. The complaint should be in writing, addressed to the medical psilocybin program, and submitted by U.S. certified mail or through the electronic system designated by the department electronic system. The department may request additional information for purposes of its own inquiry. A complaint shall include the name and contact information for the complainant; and a statement of the nature of the complaint and any individuals who may be involved. The department may, within its discretion, choose to conduct a subsequent inquiry, and may request additional information from the complainant for this purpose.

Entire section, complaints to the department

7.35.3.24 COMPLAINTS TO THE DEPARTMENT: A qualified patient or certificant may submit a complaint to the department regarding a certificant in the medical psilocybin program, or regarding any activity or concern related to the medical psilocybin program. The complaint should be in writing, addressed to the medical psilocybin program, and submitted by U.S. ~~certified~~ mail or through the electronic system designated by the department ~~electronic system~~. The department may request additional information for purposes of its own inquiry. A complaint shall include the name and contact information for the complainant, **which shall remain confidential**; and a statement of the nature of the complaint and any individuals who may be involved. The department may, within its discretion, choose to conduct a subsequent inquiry, and may request additional information from the complainant for this purpose.

SOURCE Three changes, all restorations or corrections. Confidentiality: finding M17 of `analysis/july23-rule-concerns.md`, verified against the July 9, 2026 draft, which required that a complaint include the "Name and contact information for the complainant which will remain confidential". The published section, page 15, requires the name and contact information and drops the assurance. Mail: the July 9, 2026 draft read that a complaint "should be in writing and submitted by U.S. mail or through the electronic" system; the published section requires "U.S. certified mail". Duplication: the published section reads "through the electronic system designated by the department electronic system". Section 7.35.3.24 is new in the published rule; the July 9, 2026 draft carried the same substance as an unnumbered list headed "Complaint Reporting".

Question. Do staff regain standing to complain? The July 9, 2026 draft allowed "Patients or staff" to complain; the published section allows only a qualified patient or certificant, so a healing center employee who is neither has no route. Department of Health staff and department counsel decide whether staff standing returns.

7.35.3.25 Informal administrative review of denied patient applications**CURRENT LANGUAGE**

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading and Subsection A

7.34.3.25 INFORMAL ADMINISTRATIVE REVIEW OF DENIED PATIENT APPLICATIONS: A. Administrative review: An applicant for patient enrollment whose application has been denied may request an informal administrative review from the department.

Section heading and Subsection A

~~7.34.3.25~~ **7.35.3.25** INFORMAL ADMINISTRATIVE REVIEW OF DENIED PATIENT APPLICATIONS: A. Administrative review: An applicant for patient enrollment whose application has been denied may request an informal administrative review from the department.

SOURCE Correction only. Finding N1. The heading at page 16 reads 7.34.3.25; the history note at the end of the same section reads "[7.35.3.25 NMAC, xx/xx/2026]". The substance is not amended.

Paragraph (1) of Subsection D, content and timeline

(1) Content and timeline: The administrative review shall be completed, and a decision setting forth the reasons for the decision and the evidence upon which the decision is based, no later than 15 calendar days from the date that the program receives the written request for a record review.

Paragraph (1) of Subsection D, content and timeline

(1) Content and timeline: The administrative review shall be completed, and a decision shall be issued setting forth the reasons for the decision and the evidence upon which the decision is based, no later than 15 calendar days from the date that the program receives the written request for ~~a record review~~ an informal administrative review.

SOURCE Two corrections. Grammar: finding N4 of `analysis/july23-rule-concerns.md`, verified against the published rule. The sentence at page 16 has no verb governing the decision, and is carried over verbatim from the July 9, 2026 draft. The amendment supplies the verb and changes nothing else. Terminology: the same sentence refers to "the written request for a record review", while Subsection A of the same section and Paragraph (1) of Subsection B call the proceeding an administrative review. The 15 calendar days are not amended.

Question. Who is the administrative review committee? It decides every informal review and its decision is final, but nothing in 7.35.3 NMAC or 7.35.2 NMAC constitutes it, seats it, or appoints it. Department of Health staff must say who it is before the rule is filed.

7.35.3.27 Disciplinary actions and appeal process**CURRENT LANGUAGE**

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Paragraph (8) of Subsection B, grounds for disciplinary action

(8) for certifying clinicians and practitioners: any determination by the individual's licensing body that the clinician has engaged in unprofessional or dishonorable conduct;

Paragraph (8) of Subsection B, grounds for disciplinary action

(8) for certifying clinicians and practitioners: any determination by the individual's licensing body that the ~~clinician~~ certifying clinician or practitioner has engaged in unprofessional or dishonorable conduct;

SOURCE Correction. Paragraph (8) of Subsection B, page 17, opens "for certifying clinicians and practitioners" and then refers only to "the clinician". The amendment names both classes the paragraph applies to. Carried over from the July 9, 2026 draft.

Paragraph (1) of Subsection C, persons who may request a hearing

C. Persons who may request a hearing: The following persons may request a hearing to contest an action or proposed action of the department, in accordance with this rule: (1) a certified patient; (2) a certified practitioner; (3) a certifying clinician who was certified by the department; (4) a certified facilitator; (5) a certified healing center or other approved location; (6) a certified educational program; and (7) an applicant for enrollment or certification as any of the foregoing, whose application is denied for any reason other than failure to submit a completed application or failure to meet a submittal requirement of this rule.

SOURCE Conforming amendment. Paragraph (1) of Subsection C, page 17, reads "a certified patient". A patient is enrolled rather than certified: 7.35.3.8 NMAC Subsection E, page 2, provides for "A qualified patient's enrollment", 7.35.3.22 NMAC, page 15, distinguishes "a qualified patient or certificant", and 7.35.2.7 NMAC defines "qualified patient". The subsection is reproduced in full because it is the source of the certificant definition drafted at 7.35.3.7 NMAC. Carried over from the July 9, 2026 draft.

Paragraph (1) of Subsection C, persons who may request a hearing

C. Persons who may request a hearing: The following persons may request a hearing to contest an action or proposed action of the department, in accordance with this rule: (1) a ~~certified~~ **qualified** patient; (2) a certified practitioner; (3) a certifying clinician who was certified by the department; (4) a certified facilitator; (5) a certified healing center or other approved location; (6) a certified educational program; and (7) an applicant for enrollment or certification as any of the foregoing, whose application is denied for any reason other than failure to submit a completed application or failure to meet a submittal requirement of this rule.

Subsection M, continuances

M. Continuances: The hearing examiner may grant a continuance for good cause shown. A motion to continue a hearing shall be made at least 10 calendar days before the hearing date.

SOURCE Correction. Finding N4. Subsection M, page 18, reads "The hearing examiner may grant a continuance for good cause shown". Every other subsection of 7.35.3.27 NMAC that names the presiding official calls that official the hearing officer, including Paragraph (1) of Subsection E, page 17: "All hearings held pursuant to this section shall be conducted by a hearing officer appointed by the secretary." The 10 calendar days are not amended. Carried over from the July 9, 2026 draft.

Subsection M, continuances

M. Continuances: The hearing ~~examiner~~ **officer** may grant a continuance for good cause shown. A motion to continue a hearing shall be made at least 10 calendar days before the hearing date.

Addendum A Terms this part uses and no part defines

7.35.3.7 NMAC provides in full: "The definitions in 7.35.2.7 NMAC apply to this part." Every count in the table below is produced from the text layer of the published rule and of 7.35.2 NMAC each time this document is built, section by section, so no count in it is transcribed by hand. Column headings are section numbers within each part. The first block is terms used in 7.35.3 NMAC that 7.35.2.7 NMAC does not define. The second block, shaded, is the terms 7.35.2.7 NMAC does define, shown for contrast.

Term in 7.35.3 NMAC	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	Part 3	Part 2	
educational program	.	1	.	.	.	1	.	.	.	28	.	7	.	.	3	3	13	1	2	1	3	.	.	.	.	1	3	.	68	0	
facilitator	.	1	.	.	.	1	.	.	5	5	3	1	14	5	.	.	2	15	4	3	.	.	.	.	.	1	4	.	65	0	
healing center	.	1	.	.	.	.	.	.	.	.	10	1	2	10	.	.	.	.	1	21	3	.	.	.	.	1	3	.	54	0	
certifying clinician	.	.	.	.	.	1	.	16	5	4	.	.	11	.	.	.	.	10	.	.	.	.	.	.	.	1	4	.	53	0	
other approved location	.	1	.	.	.	.	.	.	.	.	6	.	1	3	.	.	.	.	1	14	3	.	.	.	.	1	3	.	34	0	
practicum	.	.	3	.	.	.	.	.	3	4	.	1	.	.	1	.	5	.	13	2	.	.	.	.	.	.	.	.	29	0	
student	.	.	.	.	.	.	.	.	.	.	.	5	.	.	.	3	12	.	5	3	.	.	.	.	.	.	.	.	28	0	
certificant	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	2	3	3	2	.	.	1	.	11	0		
registrant	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	10	.	.	.	.	.	.	.	.	10	0	
didactic	.	.	.	.	.	.	.	.	.	4	.	.	.	.	.	.	.	4	2	.	.	.	.	.	.	.	.	.	10	0	
adverse health event	.	.	.	.	.	.	.	.	.	.	1	.	1	.	.	.	.	.	.	4	.	.	.	.	.	.	.	.	6	0	
administrative review committee	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	4	.	.	.	4	0	
graduate	.	.	.	.	.	.	.	.	.	.	.	1	.	.	.	.	2	.	1	.	.	.	.	.	.	.	.	.	4	0	
simulated patient	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	1	2	1	.	.	.	.	.	.	.	.	.	4	0	
medical psilocybin services	.	.	.	.	.	.	.	.	.	.	.	.	1	.	.	.	.	1	.	1	.	.	.	.	.	.	.	.	3	0	
equity and access fund	.	.	.	.	.	.	.	1	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	1	0	
Defined in 7.35.2.7 NMAC, for contrast																															
practitioner	.	1	.	.	.	1	.	.	5	5	3	1	12	6	.	.	2	14	6	3	.	.	.	.	.	1	5	.	66	12	
clinician	.	1	.	.	.	.	5	.	.	.	.	.	4	.	.	.	.	.	.	.	.	.	.	.	1	.	2	.	13	4	
qualified patient	.	.	.	.	.	1	.	3	.	.	2	.	4	3	.	.	.	.	3	.	3	.	1	.	1	1	.	22	6		
administration session	.	.	.	.	.	.	.	.	.	.	5	.	.	3	.	.	.	1	.	7	.	1	.	.	.	.	.	17	4		
approved location	.	1	.	.	.	1	.	.	.	.	6	1	1	3	.	.	.	.	1	15	3	.	.	.	.	1	3	.	37	2	
guide	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	0	3	

What the table shows. Sixteen terms carrying regulatory consequence in this part are defined in neither part. Six of them are defined in the draft at 7.35.3.7 NMAC, because a single provision of the published rule supplies the content: certificant, facilitator, other approved location, practicum, registrant of another approved location, and student. Ten are not, and the two largest are healing center and certifying clinician. "Guide", the one defined term in 7.35.2.7 NMAC for an individual who assists practitioners during administration sessions, appears nowhere in 7.35.3 NMAC, while "facilitator", which 7.35.2.7 NMAC does not define, appears throughout it. 7.35.2 NMAC was adopted effective June 23, 2026, so amending 7.35.2.7 NMAC is a separate rulemaking and any term this part needs has to be carried in this part.

The Act. Section 3(B) of the Medical Psilocybin Act defines a clinician as an approved health care provider licensed in New Mexico who holds a permit from the department to provide medical services to qualified patients. 7.35.2.7 NMAC defines "Clinician" in the same terms except that it reads certification where the Act reads permit, and defines "Permit" as the authorization to operate as a psilocybin producer or psilocybin testing laboratory. Section 5 of the Act exempts a

producer, a clinician and a qualified patient from arrest, prosecution and penalty. Neither the Act nor 7.35.2.7 NMAC uses the word facilitator. The consequence is stated at 7.35.3.7 NMAC in the redline above.

Addendum A, continued

The material for a definition of each term, assembled from the sources

For each term the table counts, what follows gathers the provisions of 7.35.3 NMAC, 7.35.2 NMAC and the Medical Psilocybin Act that bear on the term, quoted exactly and cited, so that a definition can be decided from assembled material rather than composed from scratch. Two of the sixteen, other approved location and practicum, are defined in the redline at 7.35.3.7 NMAC because a single provision supplies the content; they are marked below, and the source of each is quoted. Nothing below proposes language for the remaining ten, and none of the sixteen terms appears anywhere in the Medical Psilocybin Act.

healing center. The most-used undefined term, and no provision states what a healing center is. What the rule supplies: it is certified on application, 7.35.3.11 NMAC Subsection A, page 5, "An applicant seeking certification as a healing center shall submit a complete application"; it is an organization with owners, a board of directors and employees, Paragraph (11) of the same subsection; and it is a place where administration sessions occur and psilocybin products are handled, 7.35.3.14 NMAC, page 9, under which "Owners and employees of healing centers who are registered with the department may purchase medical psilocybin products from permitted producers, may possess medical psilocybin products, and may sell or otherwise administer those products to qualified patients in administration sessions conducted at the healing center or other approved locations", and "A healing center may only sell or otherwise provide psilocybin products that are obtained from permitted producers." Its operating duties are stated at 7.35.3.20 NMAC, pages 13 to 15, including the patient list at Subsection A and the reporting duty at Subsection L. Its location is what an other approved location is defined against: other approved locations "are physical locations at which medical psilocybin is intended to be consumed, that are not locations of healing centers", 7.35.3.11 NMAC Subsection B, page 6. 7.35.2 NMAC does not use the term; its nearest defined term is "Approved location", meaning "a location approved by the department for psilocybin administration sessions". The Act does not use the term and speaks instead of settings: Section 5(B)(2) makes lawful the conduct of a clinician administering or a qualified patient taking psilocybin "in an approved setting", and Section 7(A)(3) directs the department to establish "treatment protocols, including patient selection criteria, medical service standards, dosage standards and approved settings for administration of psilocybin to patients". Whether a healing center is one of the Act's approved settings is stated nowhere in this record.

certifying clinician. No provision states what a certifying clinician is, and the rule uses the bare word clinician in its scope section while using the two-word term everywhere else. What the rule supplies: the certifying clinician stands at the door of the program, since a patient application "shall be submitted by a patient and the patient's certifying clinician", 7.35.3.8 NMAC lead-in, page 1; the work is certifying patients, and "In order to certify a patient, a certifying clinician must have an actual clinician-patient relationship with the patient.", 7.35.3.13 NMAC Subsection A, page 8; an applicant must hold a license, "Documentation of current professional license to practice in New Mexico (e.g. MD, NP)", 7.35.3.9 NMAC Subsection D, page 3; and "All certifying clinicians shall complete a certifying clinician module", 7.35.3.18 NMAC Subsection B, page 11. What the other sources supply: Section 3(B) of the Act defines "clinician" as "an approved health care provider licensed in New Mexico who holds a permit from the department to provide medical services to qualified patients", and 7.35.2.7 NMAC defines "Clinician" in the same words except that it reads certification where the Act reads permit. Whether the certifying clinician of this part is the clinician that Section 3(B) of the Act and 7.35.2.7 NMAC define is stated nowhere in this record.

educational program. Regulated as a certificant, never described. What the rule supplies: it is certified on application, "An applicant seeking certification as a psilocybin educational program shall submit a complete application through the electronic system designated by the department.", 7.35.3.12 NMAC Subsection A, page 7; its application must include, at Paragraph (18) of that subsection, "A plan demonstrating how the program will ensure that enrolled students complete all required components and graduate within two years of enrollment"; and it is one of the classes that may relinquish a certification, 7.35.3.26 NMAC, page 16. 7.35.2 NMAC does not use the term. The Act does not use the term; its only training provision is Section 7(A)(2), which directs the department to establish "necessary initial and ongoing training for producers and clinicians", naming neither practitioners, facilitators, students nor programs.

other approved location. Defined in the redline at 7.35.3.7 NMAC. One provision supplies the content, which is why this term is drafted: 7.35.3.11 NMAC Subsection B, page 6, provides that "Other approved locations are physical locations at which medical psilocybin is intended to be consumed, that are not locations of healing centers." The same subsection supplies the "temporary department certification", the purposes, "if a qualified patient is unable to be physically transported to a healing center, for other reasons of medical necessity, for purposes of improving patient access in rural and frontier counties, or to enable treatment to occur in a natural environment setting", and the examples, "Such locations may include a patient's residence, or another temporary location." 7.35.2.7 NMAC defines "Approved location" but not this longer term, and the Act does not use it.

practicum. Defined in the redline at 7.35.3.7 NMAC. One provision supplies the content, which is why this term is drafted: 7.35.3.19 NMAC Subsection A, page 12, provides that an individual who seeks to become certified as a practitioner or facilitator "shall participate in supervised practice training, otherwise referred to as a" practicum. That section belongs to the practicum amendment draft, whose working draft v9 amends the hour figures in the same subsection and keeps the quoted phrase; Addendum B records the dependency. In this document's scope the word appears in the reporting duty for "Any modification,

termination, or addition of practicum site agreements", 7.35.3.15 NMAC Subsection A, page 9, in the mentoring obligation that runs "after graduation and after practicum hours are completed", 7.35.3.17 NMAC Subsection A, page 10, and in the waiver reproduced at Paragraph (1) of Subsection D of 7.35.3.10 NMAC, page 5. Neither 7.35.2 NMAC nor the Act uses the word.

didactic. The word does the work of an equivalency test in this document's scope and of an hour denomination in the practicum draft's. 7.35.3.10 NMAC Subsection A, page 4, keeps a program from another jurisdiction on the department list only while its curriculum stays equivalent to the "didactic educational requirements applicable for New Mexico certification", and removes it when "the didactic requirements of the educational program are no longer equivalent to those applicable for New Mexico certification". The module provisions of 7.35.3.18 NMAC, pages 11 to 12, state their minimums in didactic hours, and 7.35.3.19 NMAC Subsection A, page 12, opens the practicum to a student "after completing at least half of the didactic requirements and all of the simulated patient requirements of the educational requirements". No provision states which parts of a program are didactic and which are not. Neither 7.35.2 NMAC nor the Act uses the word.

adverse health event. The nearest the rule comes to content is the list of what a report of one must include. 7.35.3.20 NMAC Subsection L, pages 14 to 15, requires a healing center and a registrant of another approved location to "report any potential adverse health event associated with medical psilocybin services to the department no later than two business days" after becoming aware of it, and requires reports to include "Any challenging psychological, emotional, or behavioral reactions", "Any participant reaction requiring medical or therapeutic attention", "Any incident requiring emergency response", and "Any other incident which has a negative impact on a patient". A location's safety plan must include "adverse health event response and reporting", 7.35.3.11 NMAC Paragraph (15) of Subsection A, page 5; patient information must cover "How to report an adverse health event to the department", 7.35.3.20 NMAC Paragraph (5) of Subsection G, page 14; and a practitioner's administration records must note "Any adverse health events", 7.35.3.13 NMAC Subsection C, page 9. Whether the report-contents list is the definition is stated nowhere: it is a list of what reports include, not of what an event is. Neither 7.35.2 NMAC nor the Act uses the term.

administrative review committee. All four occurrences sit in 7.35.3.25 NMAC, page 16. "The administrative review shall be conducted by the administrative review committee.", Subsection C; "The decision of the administrative review committee is the final decision of the informal administrative review proceeding.", Paragraph (2) of Subsection D; and Subsection E provides that "Except as otherwise provided by law, there shall be no right to judicial review of a decision by the administrative review committee." Nothing in 7.35.3 NMAC or 7.35.2 NMAC constitutes the committee, states who sits on it, or says how it is appointed, and the Act does not use the term. The review note at 7.35.3.25 above records who must answer.

graduate. Four occurrences, and the fullest statement of what graduation takes is a list inside a subsection the Wilson working redline proposes to strike. An educational program must plan for enrolled students to "complete all required components and graduate within two years of enrollment", 7.35.3.12 NMAC Paragraph (18) of Subsection A, page 7; mentoring runs "after graduation and after practicum hours are completed", 7.35.3.17 NMAC Subsection A, page 10; a student using the test-out option completes "the New Mexico Module, certifications, simulated patient, and practicum requirements to graduate from the educational program", 7.35.3.17 NMAC Subsection B, page 11; and the practicum-hours waiver reaches an applicant who "Graduates from an educational program that the department certifies by December 31, 2027", 7.35.3.19 NMAC Paragraph (3) of Subsection G, page 13. Neither 7.35.2 NMAC nor the Act uses the word.

simulated patient. The rule requires the experience and never says what a simulated patient is. "A simulated patient experience of no less than 5 hours", 7.35.3.18 NMAC Subsection C, page 12; the practicum opens only after "all of the simulated patient requirements" are complete, 7.35.3.19 NMAC Subsection A, page 12; and the test-out option leaves the requirement standing, 7.35.3.17 NMAC Subsection B, page 11. Neither 7.35.2 NMAC nor the Act uses the term.

medical psilocybin services. Three occurrences, against a two-word term both other sources define. A facilitator "is authorized to work alongside a practitioner during medical psilocybin services", 7.35.3.13 NMAC Subsection B, page 8; the education requirements reach all certificants "who provide medical psilocybin services", 7.35.3.18 NMAC Subsection A, page 11; and the reporting duty covers "any potential adverse health event associated with medical psilocybin services", 7.35.3.20 NMAC Subsection L, pages 14 to 15. Section 3(D) of the Act and 7.35.2.7 NMAC define "Medical services", identically, as "services provided to a patient in an approved setting before, during and after the ingestion of psilocybin", including a preparation session, an administration session and an integration session. Whether the three-word term means the same as the defined two-word term is stated nowhere in this record.

equity and access fund. One occurrence, and no source names a fund by this name. A patient application must include "Information required for patient participation in the Equity and Access Fund (as applicable)", 7.35.3.8 NMAC Paragraph (7) of Subsection B, page 2. The Act creates two funds in Section 11, neither by this name: the "medical psilocybin treatment equity fund", to be used "to fund treatments of qualified patients who meet income requirements determined by rule of the department", and the "medical psilocybin research fund". 7.35.2 NMAC uses neither name. Whether the Equity and Access Fund is the Act's medical psilocybin treatment equity fund is stated nowhere in this record.

Addendum B Dependencies across the two drafting scopes

Provisions in these four documents that operate on, or are operated on by, a provision the practicum amendment draft in *amendments/* reaches. Nothing in these documents amends a provision in that draft's scope. Where a fix would require one, it is recorded here and left undrafted.

Provision here	Provision in the practicum draft's scope	Relationship	Handled
7.35.3.11 A(11), page 5	7.35.3.14 C, page 9	7.35.3.14 C conditions healing center owner and employee authority on being registered with the department, and no registration exists. The registration is drafted here, at the point where the healing center already files the names.	Drafted here. The practicum draft flagged 7.35.3.14 C and did not amend it, and it is not amended here either.
7.35.3.7, page 1	7.35.3.20 H(5), page 14	The definition of student drafted here is taken from the qualified student test in 7.35.3.20 H(5). The practicum draft amends that paragraph for the permit title and does not change the test.	Drafted here, sourced there. If the test changes, the definition follows it.
7.35.3.7, page 1	7.35.3.19 A, page 12	The definition of practicum drafted here is taken from 7.35.3.19 A, which provides that an individual seeking certification as a practitioner or facilitator "shall participate in supervised practice training, otherwise referred to as a" practicum. The practicum draft amends the hour figures in the same subsection and keeps that phrase.	Drafted here, sourced there. If the phrase changes, the definition follows it.
7.35.3.7, page 1	7.35.3.14 B, page 9	7.35.3.14 B authorizes facilitators to possess psilocybin products and provide them to qualified patients. Whether a facilitator is a clinician under Section 3(B) of the Medical Psilocybin Act, and so within the Section 5 exemption, governs whether that authority holds. This is the administering half of Paragraph (2) of Subsection B of Section 5 of the Act. The practicum draft reads the taking half of the same provision against Subsection H of Section 3, and its proposed 7.35.3.29 states the statutory amendment that half would need and conditions the section on it, so the two drafts read the same provision consistently.	Not drafted. Recorded at 7.35.3.7 as a question for department counsel.
7.35.3.7, page 1	7.35.3.14 A and C, page 9, with 7.35.3.20 D, page 14	The position of a student is not in doubt and is not raised in these documents. The practicum draft locates the provision of the medicine in the practitioner or the healing center, and 7.35.3.20 D provides that "students completing their practicums may be present during an administration session". The definition of student drafted here does not disturb that.	Settled in the practicum draft. Nothing here contradicts it.
7.35.3.13 B, page 8	7.35.3.14 B, 7.35.3.19 C, 7.35.3.20 H(5)	The facilitator scope of work in 7.35.3.13 B is narrower than the authority the other three provisions confer, and "direct supervision" is not defined anywhere in the rule.	Not drafted. Any amendment has to be coordinated with the practicum draft.
7.35.3.17 A, pages 10 to 11	Proposed 7.35.3.19 H, which exists only in that draft's re-lettering of 7.35.3.19	Whether the consultation requirement is stated in hours or in cases, and whether it sits at the proposed 7.35.3.19 H or at 7.35.3.17 A, is open in the practicum document for Dr. Metz, Ms. Wilson and Dr. Leeman. As published, 7.35.3.19 NMAC runs A through G and has no Subsection H.	Not drafted. 7.35.3.17 A is left exactly as published so that their answer governs.
7.35.3.10 D(1), page 5	7.35.3.19 G(4), page 13, which that draft re-letters as 7.35.3.19 K	Two waivers of the practicum hours requirement each set a 40-hour floor for applications received by December 31, 2027. For the same 40 hours of contact time, 7.35.3.10 D(1) requires "two separate group sessions" and 7.35.3.19 G(4) requires "A minimum of one group session". The practicum draft records the conflict at its 7.35.3.19 K and amends neither provision, on the ground that 7.35.3.10 is outside its scope.	Not drafted here either. Conforming one to the other is a choice between two figures already in the rule, so neither document makes it. The provision is reproduced at 7.35.3.10 above with the sharpened record, and the choice is put to the committee there.
7.35.3.16 A and C, page 10	7.35.3.18, pages 11 to 12	The third-party evaluation assesses the curriculum that 7.35.3.18 requires. Curing the evaluator conflict does not touch the curriculum, and the practicum draft's changes to the curriculum do not touch the conflict.	Drafted here. No coordination needed.
7.35.3.20 heading, page 13	7.35.3.20 heading, page 13	The heading reads 7.34.3.20. The practicum draft corrects it.	Not duplicated here. The correction stands in the practicum draft.

Addendum C Defects recorded and not drafted, and why

Gaps, contradictions, typos and drafting mistakes this draft records and does not amend, each with the reason no language is proposed.

Provision	Defect	Why nothing is drafted
7.35.3.7, page 1	Ten of the sixteen undefined terms have no definition in any source, including healing center, 54 occurrences, and certifying clinician, 53 occurrences.	Composing a definition from no source is a policy act. Addendum A assembles the material for each.
7.35.3.9 E(1) and F, page 3	A practitioner applicant must document a professional license "(e.g. PSY, LSW, LCSW)" with no stated scope of practice, and a facilitator applicant needs no license at all. The Wilson working redline comments "Also no scope here - what happened to therapy scope? It's missing - if DOH wants that, they need to include it." and "without specifying, they're setting themselves up to have athletic trainers and massage therapists apply and then not have a regulation to point to explain why that's not sufficient."	The redline raises the gap and supplies no list of qualifying licenses, and no other source in this record supplies one.
7.35.3.8 C, page 2, with 7.35.3.9 A(1), page 2	The department has 30 business days to decide a patient application and 30 calendar days to decide a certificant application.	Conforming one to the other is a choice between two figures already in the rule, and both bind the department.
7.35.3.10 D(1), page 5, with 7.35.3.19 G(4), page 13	The two 40-hour waivers set different group-session minimums, two against one.	Conforming one to the other is a choice between two figures already in the rule. The practicum draft records the same conflict and amends neither provision. The question is at 7.35.3.10 above.
7.35.3.11, page 5	The two-person universal requirement the Wilson working redline adds, which its author records did not reach the published rule.	The author records that she does not know where the provision belongs. Placing it would answer her question, which is at 7.35.3.11 B(10) above.
7.35.3.12, pages 7 to 8, against the July 9, 2026 draft	The grounds for denying an educational program application present in the July 9 draft do not appear in the published rule. Finding M9.	Restoring deleted grounds is a substantive policy decision, and this record cannot show whether the deletion was deliberate.
7.35.3.17 A, pages 10 to 11	The consultation obligation, whether it is stated in hours or cases, where it sits, and the double-count if it is stated twice.	Open in the practicum document for three named people. The question is at 7.35.3.17 A above. Addendum B.
7.35.3.17 B, page 11	The test-out price cap is a fraction of the price of "each of the educational modules", which assumes per-module pricing. The Wilson working redline strikes the subsection.	Whether the option survives at all is the committee's decision, and a restated cap is worth drafting only if it does.
7.35.3.24, page 15	The July 9, 2026 draft allowed a complaint from patients or staff; the published section allows one from a qualified patient or certificant, so staff who are neither have no route.	Restoring standing for staff is a policy decision. The confidentiality assurance the July 9 draft carried is restored in the redline above, because that text exists in a source.
7.35.3.25 C and D(2), page 16	The administrative review committee decides a denied patient application and is constituted nowhere in 7.35.3 NMAC or 7.35.2 NMAC. Four occurrences, all in this section.	Only the department can say who the committee is. The question is at 7.35.3.25 above.
7.35.3.8 D, 7.35.3.25 A, 7.35.3.27 C(7), pages 2, 16 and 17	Three provisions describe review of a denial and their scopes overlap, so a patient applicant denied on the merits may have both an informal review and a hearing. Finding M23.	The redline fixes the name of the proceeding only. Which track applies to which denial is a question of what process is owed.

Out of scope by decision. The controlled-substance number requirement for certifying clinicians, at Paragraph (3) of Subsection B of 7.35.3.8 NMAC and Paragraph (2) of Subsection D of 7.35.3.9 NMAC. The latter is reproduced in the redline above, as published, because the renumbering around it required it; it is not amended, and nothing is proposed about it.

Addendum D Mechanical defects across all twenty-eight sections

Numbering and drafting form, checked across the whole of 7.35.3 NMAC rather than only the sections this document amends. Every count and every entry is produced from the text layer of the published rule each time this document is built. The unfilled dates and history-note placeholders are the department's to fill at publication and are not defects.

Item	Count	Where	Corrected
Section headings reading 7.34.3 instead of 7.35.3	5 of 28	7.34.3.13, 7.34.3.14, 7.34.3.20, 7.34.3.23, 7.34.3.25	7.35.3.13 in the redline above; 7.35.3.14 in the practicum draft; 7.35.3.20 in the practicum draft; 7.35.3.23 in the redline above; 7.35.3.25 in the redline above
Numbering gap inside a subsection	1	7.35.3.9 D runs (1), (2), (4), (5), (6). There is no (3).	Renumbered in the redline above.
Subsections lettered "(A)" rather than "A."	4	7.35.3.11 A, and 7.35.3.14 A, B and C. Letters found: A, B, C	7.35.3.11 A in the redline above; 7.35.3.14 in the practicum draft.
Sentence with no verb governing the decision	1	7.35.3.25 D(1), page 16. Carried over from the July 9, 2026 draft.	Corrected in the redline above.
Sentence that does not complete	1	7.35.3.18 F, page 12.	In the practicum draft's scope. Corrected there.
Presiding official named inconsistently	1	7.35.3.27 M reads "hearing examiner"; the rest of 7.35.3.27 reads "hearing officer".	Corrected in the redline above.
Duplicated words	1	7.35.3.24 reads "the electronic system designated by the department electronic system".	Corrected in the redline above.
Spacing inside a heading	1	7.35.3.16 C reads "Conflict of -interest prohibitions". The July 9, 2026 draft read "Conflict-of-interest prohibitions".	Corrected in the redline above.
Class named in the lead-in but not in the operative words	1	7.35.3.27 B(8) opens "for certifying clinicians and practitioners" and then refers only to "the clinician".	Corrected in the redline above.
Patient described as certified rather than enrolled	1	7.35.3.27 C(1) reads "a certified patient".	Corrected in the redline above.

Addendum E The questions, one line each, grouped by who answers

The sixteen questions in this document, verbatim, grouped by who answers. The callout at the provision named in each row carries the basis.

AT	THE QUESTION
Department of Health staff	
7.35.3.9 D	Numbering error, or a dropped requirement?
7.35.3.10 C	Is the six months after a first denial a waiting period or a deadline?
7.35.3.11 A(11)	Is this the right home for the registration 7.35.3.14 C requires?
7.35.3.16 B(2)	Was the change from or to and deliberate?
7.35.3.25 D(1)	Who is the administrative review committee?
Department of Health staff with department counsel	
7.35.3.7	Does the Act's exemption cover facilitators?
7.35.3.8 D	Which denials get the informal review, and which the hearing?
7.35.3.24	Do staff regain standing to complain?
The Training and Education Committee	
7.35.3.7	Which of the ten undefined terms should the department be asked to define?
7.35.3.10 D(1)	Two group sessions or one, inside the same 40 waiver hours?
7.35.3.13 B	Does direct supervision mean in the room, at the location, or available?
7.35.3.16 heading and A	Adopt the engagement-fee cure at Subsection A, at C(2)(b), or both?
7.35.3.17 B	Strike the test-out option, or keep it and restate the cap?
The board	
7.35.3.11 B(9)	Keep the owner's statement for a patient's own residence, or adopt the exemption drafted here?
Dr. Metz, Ms. Wilson and Dr. Leeman	
7.35.3.17 A	Hours or cases, and stated here or at the practicum draft's proposed 7.35.3.19 H?
7.35.3.11 B(10)	Where does the two-person requirement belong?

Sources. Rule as published: *docs/documents/rules-draft-2026-07-23-published.pdf*, 19 pages, 7.35.3.1 through 7.35.3.28. Prior board-meeting draft, used for new against carried-over determinations: *docs/documents/rules-draft-2026-07-09.pdf*. Defined terms: 7.35.2 NMAC as adopted effective June 23, 2026, at *source-text/7.35.2-NMAC-adopted-2026-06-23.txt*. Statute: the Medical Psilocybin Act, Senate Bill 219, 57th Legislature, First Session, 2025, as passed; Section 1 provides that Sections 1 through 11 of the act may be cited as the Medical Psilocybin Act, and those section numbers are the ones cited here. This record does not verify any NMSA 1978 section number. Working redline: Denali Wilson, NMAC 7.35.3, July 25, 2026, tracked changes and comments as exported from the document file. July 17, 2026 transcripts, morning Advisory Board and afternoon Training and Education Committee, both labeled "UNOFFICIAL AUTO-GENERATED TRANSCRIPT. NO SPEAKER ATTRIBUTION."; no speaker is named from either transcript in this document. Inventory relied on and re-verified: *analysis/july23-rule-concerns.md*.

Method. The left column reproduces the rule as published, verbatim, with line breaks introduced by the PDF collapsed to single spaces and hyphenation introduced by a line break rejoined. Nothing else is altered. Every block was checked against the text layer of the published rule by exact contiguous match, and the build aborts if any block fails. *amendments-remainder/audit.py* re-checks every verifiable claim in these documents and exits non-zero on any failure; *amendments-remainder/AUDIT.md* is its output.

Status. Working draft v3. Not a filing, not submitted, and not adopted rule text. Nothing in *amendments/* or *docs/* was modified by this work.